

1. Administrative & Study Identification

Full Study Title: A Study to Evaluate Acalabrutinib, in Combination With the R-CHOP Standard of Care, for Previously Untreated Mantle Cell Lymphoma in Spain **Short Title/Acronym:** SOUND-MCL Study **Protocol Number:** [Internal Study ID] **NCT Number:** NCT07029737 **Trial Status:** Recruiting **Sponsor Name:** AstraZeneca **Company:** AstraZeneca **Investigational Product:** Acalabrutinib in combination with standard of care chemoimmunotherapy (R-CHOP) and maintenance Rituximab **Phase of Development:** Phase II **Medical Monitor:** [Name and Contact Information]

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To describe the efficacy of acalabrutinib in combination with R-CHOP for the front-line treatment of previously untreated Mantle Cell Lymphoma (MCL) by evaluating the Best Overall Response Rate (ORR). **Secondary Objectives:** To evaluate Time To Response (TTR), Duration of Response (DoR), Progression-Free Survival (PFS), and the safety profile of the combination regimen.

2.2 Background & Rationale To address the clinical need for improved outcomes in adult patients with previously untreated MCL who are unsuitable for autologous stem cell transplantation. This study introduces acalabrutinib, a highly selective Bruton tyrosine kinase (BTK) inhibitor, alongside the standard of care R-CHOP induction followed by rituximab maintenance, to evaluate if this combination yields an effective, tolerable, and durable response in a real-world Spanish cohort.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 55 evaluable patients **Number of Sites:** Approximately 20 sites in Spain **Estimated Study Start:** [DD-MMM-YYYY] **Estimated Primary Completion:** Up to 30 months maximum prospective follow-up from the last subject enrolled.

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Adult men or women with pathologically confirmed MCL requiring treatment, featuring chromosome translocation t(11;14)(q13;q32) and/or overexpression of cyclin D1. 2. No prior systemic anticancer therapies received, and deemed unsuitable for autologous stem cell transplantation. 3. Presence of radiologically measurable lymphadenopathy, splenomegaly, and/or extranodal lymphoid malignancy, with an ECOG performance status of ≤ 2 . 4. Willingness to follow highly effective contraception guidelines for specified periods after respective study drugs.

4.2 Exclusion Criteria 1. Subjects who are deemed by the treating physician to be unfit to tolerate the R-CHOP regimen, or whose treatment goal is tumor debulking before stem cell transplant. 2. History of central nervous system (CNS) lymphoma, leptomeningeal disease, or significant cardiovascular disease within 6 months of first dose. 3. Inadequate marrow or organ function (e.g., ANC $< 1.0 \times 10^9/L$, Platelets $< 75 \times 10^9/L$, CrCl < 30 mL/min) or active severe infections (HIV, active Hep B/C).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Induction:** Acalabrutinib 100 mg twice per day (BID) alongside 6 cycles of R-CHOP based on institutional standards. * **Maintenance:** Responders (PR or greater) receive maintenance **rituximab** (ATC Code: L01FA01; Trade Names: Rituxan, Riabni, Ruxience, Truxima, Rituxan Hycela; EMA References: [Ruxience EPAR](#), [Truxima EPAR](#)) 375 mg/m² on Day 1 of every other 28-day cycle for up to 12 additional doses. * **Monotherapy:** Following induction, non-progressing patients tolerate acalabrutinib 100 mg BID (or last tolerated dose) until disease progression. **Route of Administration:** Oral (Acalabrutinib) and IV/SC (R-CHOP and Rituximab). **Duration of Treatment:** Until disease progression or treatment discontinuation for any reason.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for chemotherapy and symptom management. **Prohibited:** Strong CYP3A inhibitors/inducers, live virus vaccinations within 28 days of study start, anticoagulation with warfarin or equivalent vitamin K antagonists within 7 days of the first dose, and concurrent participation in another therapeutic trial.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Pathological Confirmation, Baseline Imaging, Labs Induction

Cycles 1-6 (28-day cycles) | R-CHOP + Acalabrutinib
Administration, Safety Assessments, Efficacy Scans | |
Maintenance | Every other 28-day cycle | Rituximab administration
(max 12 doses), ongoing Acalabrutinib monotherapy | | **End of
Study** | Up to 30 months | Final safety labs, survival status,
exit procedures |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Best Overall Response Rate (ORR), defined as the proportion of patients achieving a Complete Response (CR) or Partial Response (PR). **Measurement Method:** Investigator-assessed best ORR as per the Lugano Classification for non-Hodgkin lymphoma (NHL).

7.2 Secondary & Exploratory Endpoints * Time To Response (TTR) * Duration of Response (DoR) * Progression-Free Survival (PFS)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection at each visit and throughout the 30-month follow-up period. A dedicated safety run-in will be performed for the first 6 patients older than 75 years (assessed after completing three induction cycles). **Serious Adverse Events (SAEs):** Typically reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** An internal committee will conduct a confirmatory assessment of sufficient efficacy for the first 10 patients treated with acalabrutinib + R-CHOP after six induction cycles.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy and Safety Set for safety run-in evaluations. **Sample Size Justification:** $N=55$ subjects. Designed as a non-indication seeking Phase II trial to describe efficacy and safety parameters adequately in the Spanish front-line setting. **Handling of Missing Data:** Handled per standard protocol definitions for oncological time-to-event analyses (e.g., right-censoring for PFS/DoR).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring throughout the 12-month recruitment period and 30-month follow-up window. **Audit Trail:** Robust clinical data management including eCRF locking, source data verification (SDV), and query resolution.

11. Study Identifiers & Governance

Primary ID: 629591880683127409 **Registry Number:** NCT07029737
Sponsor/Lead Organization: AstraZeneca **Study Title:** A Study to Evaluate Acalabrutinib, in Combination With the R-CHOP Standard of Care, for Previously Untreated Mantle Cell Lymphoma in Spain
Official Regulatory Title: The SOUND-MCL Study: A Single-arm, Open-label, Multicenter, Phase II Study of Acalabrutinib, in Combination With the R-CHOP Standard of Care, for Previously Untreated Mantle Cell Lymphoma in Spain

12. Clinical Framework (MCL Specific)

Primary Condition: Mantle Cell Lymphoma (MCL) **Disease Area:** Mantle-cell Lymphoma **Preferred Name:** Mantle cell lymphoma
Preferred UMLS Name: Mantle Cell Lymphoma, Drug Therapy **Semantic Type:** Neoplastic Process **Condition Definition:** A form of non-Hodgkin lymphoma having a usually diffuse pattern with both small and medium lymphocytes and small cleaved cells. It accounts for about 5% of adult non-Hodgkin lymphomas in the United States and Europe. The majority of mantle-cell lymphomas are associated with a t(11;14) translocation resulting in overexpression of the CYCLIN D1 gene (GENES, BCL-1). **MeSH Code:** D020522 **SNOMED ID:** 443487006 **SNOMED Hierarchy:** B-cell lymphoma (disorder); Non-Hodgkin's lymphoma (disorder); Malignant lymphoma (disorder); Lymphoproliferative disorder (disorder); Neoplasm of hematopoietic cell type (disorder); Malignant neoplasm of lymphoid, hematopoietic and/or related tissue (disorder); Malignant neoplastic disease (disorder); Neoplastic disease (disorder); Disease (disorder) **Diagnosis Threshold:** Pathologically confirmed MCL, with t(11;14)(q13;q32) and/or overexpression of cyclin D1 **Medical Methodology:** Acalabrutinib + R-CHOP Induction followed by Rituximab Maintenance **Optimization Target:** Best Overall Response Rate (ORR; CR+PR)

13. Study Procedures & Methodology

Management Pathway: 6 cycles of chemoimmunotherapy combined with targeted BTK inhibition $\rightarrow$ Maintenance phase for responders. **Education Protocol (HCP):** Administration and safety monitoring protocols for combination chemoimmunotherapy + BTK inhibitors. **Education Protocol (Patient):** Contraception compliance (highly effective forms of contraception during study treatment and up to 12 months after rituximab/cyclophosphamide, 6 months after doxorubicin, 30 days after vincristine, and 2 days after acalabrutinib; sperm banking counseling), swallowing assessments, and medication adherence. **Quality Audit Mechanism:** Age-stratified safety run-in (>75 years) and early efficacy confirmation gateway (first 10 patients).

14. Observation & Follow-Up Schedule

Total Duration: Maximum prospective follow-up of 30 months from the last subject in. **Onsite Visit Cadence:** Every 28 days corresponding with treatment cycles (Induction & Maintenance). **Remote Monitoring Frequency:** [Standard sponsor schedule, e.g., Q4W during monotherapy] **Checklist Review Interval:** After induction (Cycle 6) and periodically for survival/disease progression tracking.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					